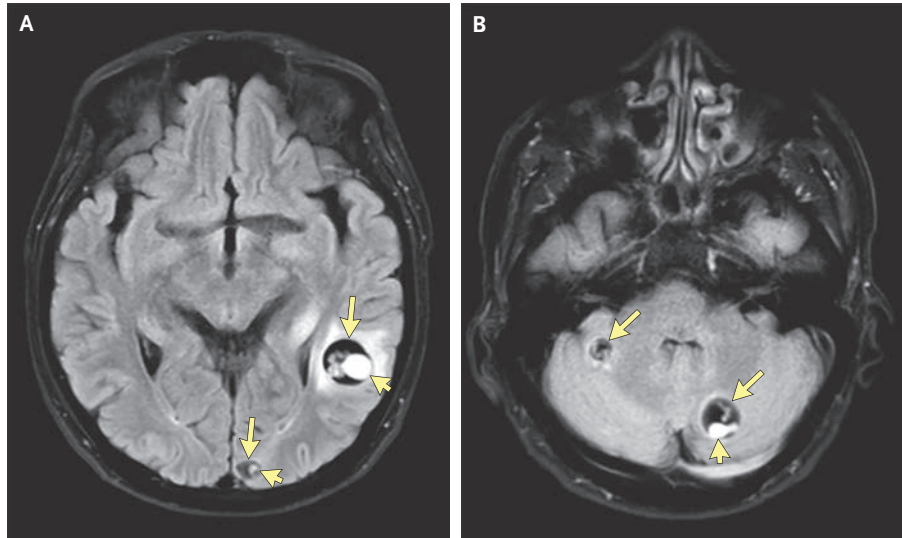


IMAGES IN CLINICAL MEDICINE

Neurocysticercosis



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A PREVIOUSLY HEALTHY 52-YEAR-OLD WOMAN PRESENTED TO THE EMERGENCY department with a 4-month history of headaches and visual hallucinations and a 1-month history of incoherent speech. On physical examination, the patient was awake and alert but unable to follow simple commands. Her speech was fluent but nonsensical, with paraphasic errors and impairment in naming and repetition. Computed tomography of the head revealed several hypodense lesions with internal hyperdensities scattered throughout the brain. Electroencephalography showed posterior temporal interictal epileptiform discharges from the left side. Results of an analysis of cerebrospinal fluid were normal. Magnetic resonance imaging of the brain revealed cystic lesions (arrows) in the temporal and occipital lobes on the left side (Panel A, fluid-attenuated inversion recovery [FLAIR] image), in the cerebellar hemispheres (Panel B, FLAIR image) and frontal lobes on both sides, and in the parietal lobe on the right side. The cysts contained bright, eccentric nodules (arrowheads) representing the scolices (anterior end) of tapeworms, a finding known as the “dot-in-hole” sign that is pathognomonic for neurocysticercosis. A diagnosis of parenchymal neurocysticercosis was made. Treatment with praziquantel, albendazole, and adjunctive glucocorticoids was given, along with levetiracetam for suspected seizures. At follow-up 1 month after starting treatment, her symptoms had abated, and repeat imaging showed smaller cysts without scolices.

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